

DIABETES: ROUTINE CARE

Give urgent attention to the patient with diabetes and any of:

- Chest pain → 37.
- Fitting → 19.
- Decreased consciousness, drowsiness
- Confusion or unusual behaviour
- Weakness or dizziness
- Shaking
- Sweating
- Palpitations
- Rapid deep breathing
- Nausea or vomiting
- Abdominal pain
- Thirst or hunger
- Temperature $\geq 38^{\circ}\text{C}$
- Dehydration: dry mouth, poor skin turgor, BP < 90/60, pulse ≥ 100

Check random fingerprick glucose:

Glucose < 4 with/without symptoms

- If alert: give **glucose**¹ 5mL/kg orally. If unable to take orally, give instead **glucose**¹ or **dextrose 10%**² 5mL/kg via nasogastric tube.
- If decreased consciousness: give **dextrose 10%**² 5mL/kg IV. If known alcohol user, give **thiamine** 100mg IM/IV before dextrose.
- Recheck glucose after 15 minutes: if still < 4, give further 2mL/kg. For IV: once glucose ≥ 4 , continue **dextrose 5%** 1L IV 6 hourly.
- Identify cause and educate about meals and doses → 131.
- If incomplete recovery or on glimepiride, glibenclamide or insulin, refer same day.

Glucose ≥ 11.1 with symptoms

- Give **sodium chloride 0.9%** 20mL/kg IV over the first hour, then 10mL/kg/hour thereafter. Stop if breathing worsens.
- If referral delay > 2 hours: give **short-acting insulin** 0.1 unit/kg IM (not IV)³.
- Refer urgently.

Glucose ≥ 11.1 without symptoms

Check urine for ketones.

Ketones present

No ketones

Give routine diabetes care below.

Assess the patient with diabetes not needing urgent attention:

Assess	When to assess	Note
Symptoms	Every visit	Manage symptoms as on symptom pages. Ask about chest pain → 37 and leg pain → 65.
Depression	At diagnosis and if control poor	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either → 143.
Alcohol/drug use	At diagnosis and if control poor	In the past year, has patient: 1) drunk ≥ 4 drinks ⁴ /session, 2) used drugs or 3) misused prescription or over-the-counter medications? If yes to any → 142.
BP	Every visit	If known hypertension → 133. If not, check BP: if $\geq 140/90$ → 132.
BMI and waist circumference	• Weight: at every visit • BMI, waist circumference: at diagnosis	• BMI = weight (kg) ÷ height (m) ÷ height (m). • Aim for BMI ≤ 25 and waist circumference < 80cm (woman) or < 94cm (man). If trying to lose weight: check BMI/waist circumference 3 monthly.
Eyes	At diagnosis, yearly and if visual problems	Check visual acuity and fundoscopy. If visual problems, cataracts or retinopathy, refer.
Feet	At diagnosis, yearly and more often if problems	Check for pain, pulses, sensation, deformity, skin problems. For foot screen and foot care education → 66.
Family planning	Every visit	Assess patient's contraceptive needs → 154. If pregnant or planning pregnancy, refer for specialist care.
Glucose	Every visit	If fasting glucose > 8 or non-fasting glucose taken 2 hours after eating > 10, step up treatment → 131.
HbA _{1c} (glucose control over past 3 months)	• Yearly if HbA_{1c} $\leq 8\%$ • 3 months after treatment change	• If HbA_{1c} $\leq 8\%$: diabetes controlled, continue same treatment for diabetes. • If HbA_{1c} > 8%: diabetes uncontrolled, if adherent, step up treatment → 131. If not adherent, give support and repeat HbA_{1c} after 3 months.
Urine dipstick	At diagnosis and yearly	• If protein, start enalapril if not already on it → 131. • If no protein and not on enalapril, send urine to lab for albumin/creatinine ratio. If ratio > 3, start enalapril → 131.
Creatinine (eGFR)	• At diagnosis, then yearly • If on enalapril: at baseline and 4 weeks⁵ • If eGFR < 60: 3-6 monthly	• Give age and sex on form. If eGFR < 60, discuss with doctor. If eGFR < 30, refer. • If creatinine increases by > 20%, stop enalapril and refer to doctor.
Potassium	If on enalapril: at baseline, 4 weeks ⁵ , then yearly	If potassium > 5.0, avoid/stop enalapril and refer to doctor.
Lipids	At diagnosis	Check fasting total cholesterol, triglycerides, HDL/LDL. Assess CVD risk → 127. If total cholesterol > 7.5 or triglycerides > 10, refer/discuss.

¹Three teaspoons sugar (15g) in 1 cup (200mL) water. ²If dextrose 10% unavailable: mix 1 part **dextrose 50%** to 4 parts water to make a **dextrose 10%** solution. ³Avoid IV insulin as it may cause low potassium and heart dysrhythmia. Avoid using an insulin needle to give IM insulin. ⁴One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ⁵If eGFR < 60, repeat instead at 2 weeks.